VCRSoft Consultation Transcript

Company: Riverbend Physical Therapy

Business type: Physical Therapy Clinic

Engagement: 47

VCRSoft

Hi Dana — I'm an AI consultant that can help you leverage AI tools and solutions to grow Riverbend Physical Therapy. Let's start: how many sites do you operate, and how many clinicians, treatment rooms, and specialties do you have across them?

YOU — Turn 1

Two sites. The flagship is in Asheville — we opened there in 2014 — and we added a second clinic in Hendersonville in 2021. Across both we have nine clinicians: seven physical therapists and two PTAs. Asheville has five treatment rooms plus an open gym area; Hendersonville has three rooms. Specialty-wise it's orthopedic and manual therapy as the core, plus sports medicine, pelvic health, and vestibular rehab, and we do dry needling within the ortho work.

VCRSoft

A four-line specialty mix on nine clinicians gives you real options. Of those lines, which one carries the largest share of your visits today — general ortho and manual, or one of the specialty programs?

YOU — Turn 2

Ortho and manual therapy is still about seventy percent of visits across both sites. Sports is folded into that largely. Pelvic health is small in headcount but it's our fastest growing line, and vestibular is me — maybe five percent of visits.

VCRSoft

So the economic engine is ortho, with pelvic health as the emerging line. How does your payer mix break down across those visits — commercial, Medicare, workers comp or auto, and cash?

YOU — Turn 3

Roughly forty-five percent commercial, thirty Medicare, fifteen workers comp and auto, and about ten percent cash pay. The cash share is mostly pelvic wellness packages, dry needling visits for people who've used their visits, and a small membership program we started.

VCRSoft

A ten percent cash slice is meaningful optionality. In a typical month, how many completed visits does the practice deliver across both sites?

YOU — Turn 4

About six hundred fifty completed visits a month, give or take. Asheville does around four hundred thirty of those and Hendersonville around two hundred twenty.

VCRSoft

That's a healthy two-site volume. How many of those start as brand-new patients — how many initial evaluations do you book and complete in a typical month?

YOU — Turn 5

Around ninety new evaluations a month combined. Somewhere in the high eighties to low nineties. Pelvic health evaluations are maybe eight to ten of those, and there's a wait for those.

VCRSoft

Noted — a wait on pelvic evals is a thread I want to pull later. You mentioned a membership program inside the cash ten percent — what exactly is that offer, and what does it include?

YOU — Turn 6

It's called Fit-for-Life — ninety-nine dollars a month for two supervised group sessions a week plus a monthly check-in with a therapist. We launched it in 2024 mostly for discharged patients who wanted to keep going. Eleven members right now. We haven't promoted it in any serious way.

VCRSoft

Eleven members with essentially no promotion says there may be untapped demand there. Beyond the nine clinicians, who else is on the team handling front desk, billing, and operations?

YOU — Turn 7

Jessica is our office manager at Asheville — she runs schedules, staffing boards, and basically the whole daily operation. Each site has one front desk person. Marta works remote doing insurance verification and prior auth support. That's thirteen of us total including me.

VCRSoft

So a lean four-person support layer across two sites, which makes front-desk time precious. How is your own week split between treating patients and running the business?

YOU — Turn 8

I still treat about twenty-five hours a week — mostly vestibular and a small ortho caseload — and the rest of the week, fifty-five hours total roughly, goes to the business. Realistically the treating half is fixed in my head for now because the vestibular patients come specifically for me.

VCRSoft

Half a clinical schedule on the owner is exactly the kind of constraint that shapes what growth is possible. What does growth actually look like for you — if we're sitting here in two or three years and it worked, what's different?

YOU — Turn 9

Two things. One, we've expanded the Hendersonville suite into the space next door — it's adjacent, the landlord has offered it — so that site runs five or six rooms instead of three. And two, pelvic health is a two-therapist line with its own little following, not one overloaded therapist. Longer term, a third location north of Asheville is the dream, but I won't pretend that's a plan yet.

VCRSoft

Adjacent expansion plus a second pelvic therapist is a concrete, fundable picture. What's the most consequential problem standing between you and that picture right now?

YOU — Turn 10

Capacity at the wrong hours. We're effectively full when patients can actually come — six-thirty to nine in the morning and four to seven in the evening are packed. Midday we have room. So we end up with a waitlist of people who all want prime time, and we occasionally send prime-time demand to other clinics, which I hate.

VCRSoft

Prime-time scarcity with midday slack is a very specific shape of problem — it means the constraint is the schedule, not demand. How big is that waitlist today, and what happens to the people on it?

YOU — Turn 11

Thirty-five or so patients on the list at any time, and honestly eighty percent of them only want early or late slots. Some we place within a week or two, some get tired of waiting. I'd guess a handful a month book somewhere else and we never see them again.

VCRSoft

A waitlist that's eighty percent prime-only means a midday cancellation almost never gets refilled from it. You mentioned a number you'd want to hit — what's the actual growth target in dollars or visits, and by when?

YOU — Turn 12

From about nine hundred thousand a year in collected revenue today to one and a quarter, one point three million within twenty-four months. That's the expansion running at full rooms plus one more clinician. In visits that's roughly eight hundred fifty a month.

VCRSoft

About forty percent revenue growth in twenty-four months, hinged on two extra rooms and a hire. What specific conditions or gates would have to be true before you'd sign that expansion — the criteria, not the wish?

YOU — Turn 13

I've got informal gates. Collected revenue at eighty-five thousand a month or better for ninety days. A signed clinician hire before buildout starts — I won't build rooms and then starve them. Prime-time utilization already above eighty-five percent, which we're at. And four months of operating cash in reserve, which honestly we're short on — we hold about ninety-five thousand and I'd want a hundred twenty.

VCRSoft

Those are real gates, and the cash one plus the hire one are the binding pair. You mentioned the landlord's offer on the adjacent space — what exactly is on the table, and what's the deadline attached to it?

YOU — Turn 14

Twelve hundred square feet directly adjacent to Hendersonville, which would add two more treatment rooms. The landlord needs an answer by the end of first quarter 2027 — that's when he starts marketing it elsewhere if we pass. Buildout estimate is eighty-five to a hundred ten thousand dollars depending on plumbing. Five months of runway on the decision, give or take.

VCRSoft

A hard deadline in five months with a six-figure buildout makes this a live decision, not a someday one. How is the hiring side actually going — the signed hire your own gates require?

YOU — Turn 15

That's the painful one. Two open PT requisitions — one in Asheville since March, one in Hendersonville since June. We've used recruiters and the board postings. Historically our searches take three to five months, and the last two hires took five. For pelvic health specifically I'd need someone experienced, because Nadia can't mentor a new grad alone while carrying a full caseload.

VCRSoft

So the hire gate is stuck exactly where growth usually stalls — specialized clinicians are scarce and searches are slow. If the first-quarter window passes without the hire or the cash, what happens?

YOU — Turn 16

He re-tenants the suite, most likely to another medical user, and we're back to hunting for space in a small market with almost nothing adjacent. That sets the Hendersonville expansion back twelve to eighteen months, and honestly it takes the wind out of the team. That's the clock I'm racing.

VCRSoft

Understood — a twelve-to-eighteen-month setback against a twenty-four-month goal. Where do new patients actually come from today — walk me through the channels and roughly what share each contributes?

YOU — Turn 17

Physician referrals are still king — call it forty-five to fifty percent of new patients. Past patients and word of mouth another twenty to twenty-five. Google search and our website maybe eighteen. Direct access self-referrals — people who just call us — around eight. The last few percent is attorneys and employers for the workers comp cases.

VCRSoft

A referral-led engine with a real digital tail — that mix is common and workable. How many physician referrers are actively sending you patients, and which sources matter most?

YOU — Turn 18

Twelve to fifteen practices send us patients in any given month. The big three are Asheville Orthopedic Associates, Blue Ridge Spine Center, and two primary care groups — one in Asheville, one in Hendersonville. On the workers comp side, two employers and an attorney send repeat business.

VCRSoft

Twelve to fifteen active referrers is a real bench, but benches leak. Can you actually see which referrers have gone quiet over the last six months — is referral tracking reliable enough to spot that?

YOU — Turn 19

Honestly, no, not reliably. The source field in WebPT is free text, so the same doctor gets typed in five ways — doctor's name, practice name, just physician, sometimes left blank. We can't run a clean report on it. Case in point: Dr. Patel, a sports med doc who used to send us six to eight patients a month, has been sending one or two. I only figured that out from a hallway conversation at a conference three months after it started.

VCRSoft

A quiet referrer costing you four to six patients a month for a quarter is exactly the kind of leak that never announces itself. Do you give referring physicians anything back — progress updates on their patients during or after a plan of care?

YOU — Turn 20

Patchy. Therapists fax a discharge summary if they remember, and Medicare plans of care go back to the referring provider because they have to. But a proper progress touchpoint mid-episode — no, we don't do that systematically. Some docs have told us we're the only clinic that even faxes anything, which tells you the bar.

VCRSoft

So the referral relationship runs on goodwill with almost no structured communication. What does your review presence look like — Google review counts and ratings, and how consistently are discharged patients asked?

YOU — Turn 21

Asheville has a hundred forty-eight reviews at four point eight. Hendersonville has forty-one at four point seven. Asks are ad hoc — front desk mentions it when someone graduates and they remember. I'd guess a third of discharges actually get asked.

VCRSoft

Strong ratings, inconsistent ask — that's a common and fixable gap. North Carolina allows direct access — how does that actually play out for you, do patients come without a referral?

YOU — Turn 22

They can, and a few do — mostly pelvic patients who've researched us, and athletes. But most still arrive with a referral because their insurance wants one, and Medicare requires a referring provider on the plan of care anyway. So direct access is real but it's the smallest voluntary channel we have.

VCRSoft

Direct access as a niche channel, then. Of the inquiries that never become patients at all, what reasons do you actually hear from them?

YOU — Turn 23

A few recurring ones. We didn't call them back fast enough — evenings and weekends especially. Price shoppers for cash services who balk. People who went where their doctor told them to go regardless of what they wanted. And insurance confusion — people who thought we were out of network or didn't understand their copay and just gave up.

VCRSoft

Slow callbacks and insurance confusion are both process failures you can attack. How quickly do you respond to an inquiry that arrives in the evening or on a weekend — what happens to calls that hit voicemail?

YOU — Turn 24

Phones roll to voicemail after five-thirty and on Saturdays. Voicemails get returned next business day. The website form lands in an inbox that Jessica checks in the morning. So a seven p.m. caller on Thursday hears back Friday, if we're on it. We've absolutely lost evening callers who booked somewhere else by then.

VCRSoft

Next-business-day callbacks against same-evening competitors is a measurable leak. What are you spending on marketing — ads, sponsorships, anything — and can you tie any of it to booked evaluations?

YOU — Turn 25

Fourteen hundred a month on Google Ads, Hendersonville only, running since January 2025. We shut the Asheville campaign off at the end of 2024 because I couldn't tell whether it was working. Add three hundred fifty or so in local sponsorships. Attribution is the honest weakness — the ads maybe drive ten to fifteen calls a month, but I cannot tie a call to a booked evaluation, so I'm spending on faith.

VCRSoft

Spending without attribution means you can't double down or cut — both decisions need the same missing data. Once a new patient does book, how many start a plan of care, and how many visits does a typical episode run?

YOU — Turn 26

Ninety percent or so of completed evaluations start a plan of care. Typical ortho episode is about nine visits, pelvic is around eight, vestibular more like six. Payer mix moves those a bit — Medicare plans run shorter.

VCRSoft

Healthy conversion at the front of the funnel. Where does it fray later — how many patients actually complete their prescribed plan, and where does drop-off concentrate?

YOU — Turn 27

About two-thirds complete the plan. Drop-off clusters around visits four to six — people feel better, copays add up, rides get hard to arrange. We find out about drift when someone no-shows and the front desk flags it, or a therapist notices a gap in their schedule. WebPT has a report for it but nobody runs it on a schedule. It's reactive.

VCRSoft

Reactive drift-spotting means the intervention lands after the patient is already gone. After discharge, what happens — do you reactivate past patients, and what happened with that reactivation idea you tried?

YOU — Turn 28

Almost nothing systematic. In 2024 we emailed about three hundred past patients with a check-in offer and four booked — which I actually thought was fine for an email blast, but we never repeated it. Fit-for-Life just sits there with eleven members because nobody owns it. Discharged patients walk out the door and mostly we hope they remember us.

VCRSoft

Four bookings from one email says the list has life in it — an unowned asset. Let's shift to the money: what's your monthly collected revenue, and what margin are you actually clearing?

YOU — Turn 29

About seventy-five thousand a month collected, so call it nine hundred thousand a year run rate. Margin on paper is thirteen to fifteen percent depending on the month. It feels thinner than that because my own pay is inside the expense line.

VCRSoft

Nine hundred thousand collected with mid-teens margin — workable, but the expansion gates you set need roughly ten thousand more a month. Help me reconcile that: with six hundred fifty visits, what does an average visit actually net you?

YOU — Turn 30

About a hundred fifteen dollars average net across the practice. Medicare pulls it down, cash pulls it up. Hendersonville runs lower — maybe a hundred five — because it's Medicare-heavier, Asheville closer to a hundred twenty.

VCRSoft

That arithmetic holds — seventy-five thousand over six hundred fifty visits is about a hundred fifteen. Before a patient ever reaches a visit, what happens between booking and first appointment — walk me through intake and verification and how long it takes?

YOU — Turn 31

Marta verifies benefits and authorizations for new patients. Done properly it's twenty to thirty minutes per patient — she logs into the payer portal, checks PT benefits, copay, visit limits, auth requirements. She batches it twice a week, which means same-day new patients sometimes get a rushed version or the front desk does a quick check.

VCRSoft

Twenty to thirty minutes per new patient, batched, with a rushed fallback — that's where errors breed. Where do intake or verification errors actually show up, and when are they caught?

YOU — Turn 32

Three times last quarter we quoted benefits from the wrong service — the payer portal returned their gym or wellness program benefit, not outpatient PT, and the front desk quoted a twenty-dollar copay that turned out wrong. Workers comp is the other sore spot: two or three times a month the authorization comes in late, sometimes after the first visit. Those get found at the first visit or after, and then it's rework, patient confusion, and occasionally a denial we have to appeal.

VCRSoft

Wrong-benefit quotes and late comp authorizations are specific, recurring, and found too late — that's a classic pre-visit data problem. Once visits happen, how does billing perform — denials, collections, who handles it?

YOU — Turn 33

Billing is outsourced — they take three percent of collections. First-pass denial rate is six to eight percent. Accounts receivable sits around thirty-four days. The denial reasons we see most: eligibility and authorization problems — which connect to what I just described — documentation, meaning the plan of care wasn't signed in time, and coding, usually timed versus untimed unit disputes.

VCRSoft

Half your denial categories trace straight back to the pre-visit verification step — the problem is upstream of the biller. What software and systems hold all this together today?

YOU — Turn 34

WebPT is the spine of everything — records, scheduling, the billing workflow. Google Workspace for email and docs. Phones are basic VoIP with voicemail, nothing smart. The waitlist is literally a spreadsheet plus front desk phone calls. No texting tool of any kind — reminders go out as WebPT automated emails, which patients ignore at a remarkable rate.

VCRSoft

WebPT as the system of record with everything around it manual is a clear picture. You said the referral source field is messy — beyond that, how trustworthy is the data in WebPT for actually running the business?

YOU — Turn 35

Schedule and visit data is solid — if WebPT says the visit happened, it happened. The source field is the garbage-in part, and anything downstream of it, like referral performance, is a guess. Insurance data is as good as whoever typed the card in. So: solid core, unreliable edges.

VCRSoft

Solid core, unreliable edges — that decides which automations are even possible. Paint me the capacity picture by time of day — how full are you at each part of the day, and how do open slots actually get filled?

YOU — Turn 36

Prime time — six-thirty to nine a.m. and four to seven p.m. — runs ninety-two to ninety-five percent full. Ten a.m. to two p.m. is fifty-five to sixty percent. When a cancellation comes in, the front desk works the spreadsheet waitlist by phone. Prime-time cancels, we maybe fill half the same day. Midday cancels almost never fill because everyone on the list wants early or late. The rooms just sit empty.

VCRSoft

So every week you're leaving booked-but-lost prime slots plus systematically empty midday hours — that's real margin evaporating. What about AI and automation — what are you already using, including anything for documentation or notes?

YOU — Turn 37

Two things. I personally use ChatGPT for marketing copy and job postings — nothing with patient data. And we ran a real scribe pilot in March and April this year — the Freed scribe, two clinicians, six weeks. Right now documentation eats forty-five to sixty minutes per clinician per day, after the last patient, and it's the number one morale complaint I hear.

VCRSoft

A scribe pilot that actually ran is a live lead — most clinics only talk about it. What happened with it — why did it stop, and what did you learn?

YOU — Turn 38

It genuinely worked. Clinicians saved thirty-five to forty-five minutes a day, easy. But the notes needed real editing at first — five to eight minutes each before they were signable. And it was copy-paste out of the scribe's portal into WebPT, which everyone hated but tolerated. We signed their BAA. What we never did was set up a note audit — I wanted someone spot-checking notes and I never made it happen. Then my father got sick in May and I stepped away for three weeks, the pilot just lapsed, and nobody restarted it. Honestly we never captured a single measurement either. No baseline, no after numbers.

VCRSoft

So the pilot proved the time savings but died from missing ownership, missing measurement, and a copy-paste workflow — all three fixable. On money: what monthly tool budget and one-time setup budget would you actually approve if the return was clear?

YOU — Turn 39

Five hundred to eight hundred a month in ongoing tools I can stomach, especially next to what a half-time hire costs. One-time, five to ten thousand if it's genuinely setup — I'm not paying that monthly. And whatever it is, it has to not add work to Jessica for the first month, because she has no slack.

VCRSoft

A real budget with a real constraint — Jessica's bandwidth is as binding as the dollars. Suppose we pilot something: what measurable change inside ninety days would make you keep it, and who would track that?

YOU — Turn 40

Keep-it numbers: no-shows down three points from the eleven to twelve we run now. Verification under ten minutes a patient. Documentation under thirty minutes a day per clinician. And at least sixty percent of open slots filled within two business days off the waitlist. Jessica would pull the WebPT reports — she's capable, she just needs someone to hand her the exact list — and I'd review monthly. That's a fair standard. Anything that can't hit one of those in ninety days doesn't survive.

VCRSoft

A crisp scoreboard — that's rarer than you'd think. Let me pressure-test the picture before we go deeper: is demand genuinely exceeding your capacity, or could the prime-time crunch be hiding a demand problem in parts of the business?

YOU — Turn 41

In Asheville, demand genuinely exceeds prime-time capacity — the waitlist and the pelvic wait prove it. Hendersonville is different: the referral base there is thinner, maybe five or six active referrers versus a dozen in Asheville, which is partly why the Google Ads test happened there. So: capacity-constrained in Asheville, demand-limited in Hendersonville, and pelvic health is capacity-constrained everywhere.

VCRSoft

A split diagnosis — that's important, because the two sites need different fixes. Stay on pelvic for a moment: how deep is that waitlist, and what is the wait actually costing you?

YOU — Turn 42

Fourteen people waiting right now, four to six weeks out. Nadia is the only pelvic-trained therapist, her pelvic blocks are full, and we turn away four to six new pelvic patients a month — some drive across town to our competitors, some wait. Those are eight-visit episodes at strong reimbursement, largely cash-friendly. The demand is there; the clinician isn't.

VCRSoft

Turning away four to six episodes a month at eight visits each is close to full-time clinician's worth of unserved demand. What would hiring that second pelvic therapist actually take — profile, cost, timeline?

YOU — Turn 43

Experienced pelvic PT, not a new grad — Nadia can't mentor and carry a full caseload. Market salary is seventy-eight to eighty-five thousand, and I'd add a mentorship or training budget on top. Realistically a three-to-five-month search because everyone recruits the same small pool. If I had her tomorrow, the waitlist alone fills her schedule inside a month.

VCRSoft

So pelvic growth is gated by a hire that takes months — which makes everything you can do without hiring more valuable. Back to the scribe pilot: what stopped it from restarting, and who would own it if it did?

YOU — Turn 44

Nothing stopped it — it just nobody's job. I was the sponsor, I went dark for three weeks, and the clinicians slid back to typing their own notes because that's the default. The other honest factor: Mark, my most senior PT, was lukewarm the whole time — his line is that scribes make lazy clinicians. Nadia loved it. Without me pushing, the skeptical voice wins by default.

VCRSoft

Champion versus skeptic with the sponsor distracted — that's an adoption risk you've already lived. Digging into trust and quality: what would a note-audit process need to look like for you to be comfortable, and who could actually do it?

YOU — Turn 45

The reviewing therapist reads the note before signing regardless — nobody signs blind. Beyond that, I'd want a weekly spot-check of five notes, and Dana-time is the only honest answer for who does it, maybe twenty minutes a week. Start with two clinicians again — Nadia and me — before anyone touches Mark's schedule. Cost was ninety-nine per clinician per month, so two of us is two hundred a month, which is noise against forty-five minutes a day.

VCRSoft

A bounded restart with a named auditor and a real metric — that's a credible design. What about the workflow problem — is the copy-paste into WebPT the only option, has anyone checked integration?

YOU — Turn 46

We never asked WebPT about integration — not once. It was copy-paste from the scribe's portal into the note field, which works but adds friction and, honestly, transcription risk. Whether WebPT has a scribe integration or an API path is a verify item for me, and I'd want that answered before the restart, not after.

VCRSoft

Right answer — verify the integration before relaunching. Switching to the scheduling leak: walk me through what happens when a prime-time cancellation lands — step by step, who does what?

YOU — Turn 47

Patient calls or the front desk notices a gap in WebPT. Front desk opens the spreadsheet waitlist and starts calling down the list — but only the people flagged for that time of day. Prime-time cancels, she reaches someone maybe half the time the same day. Midday, almost nobody on the list wants it, so those slots die quietly. Meanwhile the next patient on the list might have said yes to a text in thirty seconds. It's all manual phone tag.

VCRSoft

Phone-tag triage for every cancellation, one at a time — that's the mechanism behind the fill rate. What's your actual no-show and late-cancellation rate, and what reminders run today?

YOU — Turn 48

Combined no-show plus late cancel runs eleven to twelve percent. Reminders are WebPT's automated emails — no texts at all. If a patient doesn't check email, there's no confirmation loop, no two-way anything. I suspect half our no-shows are plain forgetters who would have replied yes to a text.

VCRSoft

Email-only reminders against eleven to twelve percent leakage — the texting gap is measurable on day one. For a waitlist and reminder tool to earn its keep, what proof standard would you set, and what would make you kill it?

YOU — Turn 49

Same standard I gave you: sixty percent of open slots filled within two business days, no-shows down three points. Kill it if opt-outs run hot — if patients are annoyed by the texts, we've traded a scheduling problem for a reputation one. And it has to actually talk to WebPT's calendar, not be another island I check separately.

VCRSoft

Integration, annoyance risk, and a number to hit — that's a fair trial. On the referral side: if you could see referrer volume cleanly, what would you actually do with it — what does the Patel situation call for?

YOU — Turn 50

First, catch it early — a monthly report by referrer would have shown Dr. Patel dropping in four to six weeks instead of the three months it took a hallway conversation. Second, act on it — a call or coffee, find out whether it's something we did or something on their end. And for the top ten referrers, send something useful quarterly, like a simple note on how their patient progressed. Right now we do none of that systematically.

VCRSoft

Catch it in weeks, act on it, feed the top ten — a referral system instead of referral luck. One more pre-visit thread: could benefits verification ever run itself, or is it hopeless across all those payer portals?

YOU — Turn 51

It's manual today — Marta lives in payer portals, and every payer's is different. What I'd want is simple: benefits checked automatically before we schedule, not the day before the visit. Roughly ninety new patients a month would benefit, plus the workers comp auth tracking. Whether that needs some clearinghouse hook into WebPT, I don't know — that's exactly the kind of integration question I'd need answered before I believed it.

VCRSoft

Ninety patients a month at twenty to thirty minutes each is thirty to forty-five staff hours — real money even before the error reduction. Last depth thread: reviews and reactivation — why did Fit-for-Life stall, and what would it take to revive the ask?

YOU — Turn 52

It stalled because nobody owned it — same disease as the scribe pilot, I'm noticing a pattern. We never put the ask into the discharge workflow, never emailed the list again after 2024, and I've got maybe eighteen hundred past-patient emails sitting in WebPT doing nothing. Eleven members without trying tells me the offer isn't broken — the distribution is. If discharge included a review ask and a Fit-for-Life pitch every single time, something would move.

VCRSoft

The pattern has a name — good initiatives with no owner and no measurement. Before we talk priorities: what compliance, privacy, or consent limits would any of this have to respect — scribes, texting, patient outreach?

YOU — Turn 53

HIPAA is the frame — anything touching patient communication needs a BAA, full stop. The scribe pilot had one signed, which was right. Consent wording matters too: if we text patients or use a scribe in the room, the intake paperwork has to say so plainly. And no patient information in any tool without a BAA, including my ChatGPT habit — that stays marketing-only. State board expects notes signed timely regardless of how they're drafted.

VCRSoft

Clear lines: BAA everywhere, plain consent at intake, no PHI outside the circle. Who could kill one of these tools in practice even if the numbers work — adoption risk?

YOU — Turn 54

Mark, honestly. If he digs in, the other therapists watch him and wait. Nadia is my champion and Jessica is eager, but skeptics set the pace in a clinical team. Maybe the smart move is giving Mark the note-audit role — if he owns quality, he owns the tool instead of fighting it.

VCRSoft

Converting your skeptic into the quality owner is the kind of move that makes rollouts stick. Logistically, who would own implementation, and how many hours a week can they really give it?

YOU — Turn 55

Jessica — four to five hours a week, not more. I make decisions, but mostly after hours. No IT staff; we lean on WebPT support and vendors. Whatever we add has to fit inside Jessica's five hours or it waits, because she's already carrying two sites.

VCRSoft

Jessica's five hours is the real implementation budget — noted. You treat twenty-five hours a week; what would you do with hours freed by tools rather than by you treating less?

YOU — Turn 56

Referrer relationships first — lunches with the top practices, especially in Hendersonville where we need to widen the base. Then hiring — staying on top of the two open requisitions properly. Then expansion prep — the buildout decisions, equipment lists, scheduling model for six rooms. I'm the bottleneck on all three right now; growth work only happens after six p.m.

VCRSoft

That's the leverage case in one sentence: freed owner hours go directly into the three things your gates require. Hendersonville's thinner referral base — what's the actual plan there beyond the ads test?

YOU — Turn 57

Primary care outreach, honestly — there are more family practices there we've never visited. Community events, a running club partnership, those move the needle locally. The ads may be helping but I can't prove it, which is its own problem. This is exactly why the referral cleanup matters most there — I can't grow a referral base I can't see.

VCRSoft

You can't manage what the data won't name — that ties the referral work straight to the Hendersonville goal. Quick audit question: of the numbers you've given me, which ones don't fully reconcile in your own head?

YOU — Turn 58

Three. The no-show rate — I've said eleven and twelve in the same breath because it moves month to month and I've never pinned it. The ads — fourteen hundred a month is the invoice number, but calls-to-evals is a guess. And the per-visit gap between sites — a hundred five versus a hundred twenty — is partly Medicare mix and partly a couple of discounted employer contracts in Hendersonville I didn't itemize. I should reconcile those properly.

VCRSoft

Naming your own unreconciled numbers is exactly what makes the rest credible. One more alternative to rule out: could growth come from pricing or mix instead of capacity — premium or cash programs beyond today's ten percent?

YOU — Turn 59

Maybe, and I've thought about it — performance training for runners, more pelvic wellness packages, small-group programs. High margin, cash up front. But every time I start building one, a clinic day eats it. So it's a real hypothesis, not a plan — I can't call it proven any more than the ads.

VCRSoft

Honest — hypothesis, not plan. Last question before we talk priorities: if you could move one number in this business by March — fill rate, no-shows, or completed plans — which one moves the P and L fastest, and why?

YOU — Turn 60

Fill rate, no contest. A filled cancellation is cash in the door this week with zero clinical cost — the therapist is already paid for the hour. No-shows matter but they're part of the same leak. Completed plans is the slow build. Fill rate first, everything else follows.

VCRSoft

Let me test a draft ranking. One: restart the scribe pilot with measurement — thirty-five to forty-five minutes a day saved; Mark's skepticism is the risk. Two: waitlist and cancellation texting — it attacks fill rate directly, the number you said moves the P and L fastest. Three: referral cleanup with a monthly referrer report — it protects the channel feeding half your volume. Does that order match what you would act on first, or would you change it?

YOU — Turn 61

I'd change it — swap the first two. Texting goes first: it's revenue now, Jessica can own it, and it doesn't ask clinicians to change one behavior. Scribe restart second, but redesigned — Mark as note auditor, real measurements this time, and the WebPT integration question answered before we start. Referral cleanup third: slower burn, but start the monthly report immediately because it's nearly free. Order confirmed as one,

two, three in that sequence.

VCRSoft

Texting first, measured scribe second with your skeptic as auditor, referral intelligence third — that order is defensible on value and on who has to change, which is usually the real constraint. For the scribe restart to be credible when you take it to Mark, what measurements would you need to see to call it a win?

YOU — Turn 62

Thirty minutes a day saved minimum, edits under ten minutes per note, notes signed the same day, and no uptick in patient complaints about the room. Five notes spot-audited weekly — by Mark, if he takes the role. If the WebPT integration turns out to be real, the copy-paste friction disappears and this gets easy. If not, we live with it and still make the math work.

VCRSoft

Those are auditable numbers, which is all a second pilot needs to be taken seriously. One last risk check: what would make you pull the texting tool in its first month?

YOU — Turn 63

Opt-outs running hot — if patients are annoyed, it's dead, I don't care what the fill rate does. Or the calendar sync fighting WebPT — double-bookings would burn trust with the front desk in a week. I'd also want the consent script right at intake before the first text goes out. Get those wrong and the tool costs more goodwill than it earns.

VCRSoft

Consent first, calendar sync proven, watch opt-outs — sensible tripwires. Final one: if both rollouts fit your budget, is there anything left that would still hold you back from running them together?

YOU — Turn 64

Only Jessica's hours. She can't run two rollouts in the same month — texting goes first, scribe follows in January once the dust settles. And if the WebPT integration answer comes back bad on the scribe, that one waits until it doesn't. Sequence it so nothing lands on her desk twice in the same week and I'm confident this works.

VCRSoft

Thanks — I have prepared a draft of your growth strategy report. Please review it, and tell me what you would change, correct, or push back on before I finalize it.

YOU — Turn 65

[Draft feedback] Mostly right. Three corrections and one addition. One: we have seven physical therapists and two PTAs — the draft calls all nine clinicians physical therapists. Two: the ads figure is approximate, say about fourteen hundred a month. Three: the scribe savings are from our own six-week pilot, not a vendor claim — label them that way. And the referral cleanup section reads like an Asheville play — the thinner referral base is Hendersonville's problem, so weight it there.